

我的猫现在要不要立刻去医院

写给谁：正在担心、但不确定"是不是该现在就去"的主人。不需要任何医学背景。

怎么用：从第 1 节开始，**一条一条对**。对上任何一条，就不要再往下读了——直接联系兽医。

性质：循证参考资料，**不是诊断**。它不能代替能亲手摸到这只猫的兽医。

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第 1 节 · 对上任何一条，现在就联系兽医

不要先查资料，不要等到明天早上，不要等"看看会不会自己好"。

呼吸

- 张口呼吸、伸着脖子呼吸、肚子明显一起一伏地用力
- 呼吸又快又浅，或者呼吸时有明显声音
- 舌头或牙龈发紫、发白

排尿（公猫尤其）

- 反复进猫砂盆、蹲着用力，却尿不出来或只有几滴
- 在猫砂盆里叫、舔下体、碰肚子就痛

这是本页最需要被记住的一条。尿道阻塞可以在一天之内致命，但它是**可以治的**——真正的问题在别处，见第 3 节。

突然瘫、突然不能走

- 后腿（或一条腿）突然拖着、冰凉、叫得很痛
- 脚垫颜色变白或发紫

吃与吐

- **完全不吃**（一口都不吃）
- 反复呕吐、连水都留不住
- 明显脱水、极度虚弱、站不起来

神经

- 抽搐、肌肉不停发抖、意识不清

- **突然看不见了**：撞家具、找不到食盆、瞳孔放大且不随光线收缩

中毒（接触过就算，不必等症状）

- **百合**：任何部位、任何品种、哪怕只是舔了花粉或花瓶里的水
- **含除虫菊酯／氯菊酯（permethrin）的狗用驱虫滴剂**
- 防冻液、人用止痛药（布洛芬、对乙酰氨基酚）、老鼠药

其他

- 体温明显偏低、身体摸起来发凉
- 昏倒过（哪怕已经缓过来）
- 严重外伤、被车撞、高处坠落——**即使看起来没事**

⚠️ 这份清单的正确用法。它列的是「不要等」的情况，不是「不在表上就可以等」。

你的猫、你的观察、你的直觉，都比一张通用清单更了解它。觉得不对劲就打电话——这本身永远是一个合理的决定。

📞 第 2 节 · 决定去了，路上做这几件事

1. **先打电话**，说明"我认为是急诊"和最主要的那个症状。医院可以提前准备，也会告诉你该去哪。
2. **确认这家医院现在有没有急诊能力**——很多诊所夜间不接急诊，白跑一趟就是在浪费时间。
3. **路上整理这些**（不用写得漂亮，能说清就行）：
 - 症状**从什么时候**开始，是突然还是逐渐
 - **最后一次正常吃饭、正常排尿**是什么时候
 - 正在吃的**所有药、保健品、最近换过的粮**
 - 有没有可能接触到植物、药品、化学品
 - 有旧的化验单就带上，或拍照
4. **如果猫呼吸困难，尽量减少折腾**：放进航空箱、少搬动、路上保持安静。

⚠️ **不要在家给猫喂任何人用药**。布洛芬和对乙酰氨基酚对猫有毒。

💰 第 3 节 · 一件几乎没人告诉主人、但可能改变结局的事

这一节不是急诊指引，是**在你被要求做决定时该知道的数字**。可以之后再读。

3.1 尿道阻塞：大多数死亡是决定，不是疾病本身

英国初诊数据，1108 例记录完整的公猫尿道阻塞里：

329 只 (29.6%) 在这次发病中死亡——而其中 285 只 (88.0%) 是安乐死 (Beeston 2022, PMID 35199370)

近九成的死亡，是在诊室里做出的决定，通常关于费用、复发和预后——而这三件事，往往没有人替主人量化过。

这不是在说那些决定是错的。 复发是真的，费用是真的。这是在说：**这个决定值得拿到真实的数字。**

关于复发（这是最常被提到的理由）：39 只猫的长期随访中，特发性组再阻塞 **8/22 (36%)**，尿石组 3/10 (30%)，尿栓组 3/7 (43%)；再阻塞是安乐死最常见的理由，占 **8/39 (21%)** (Gerber 2008, PMID 17719255)。

⚠️ n=39、转诊人群、2008 年——把它当数量级，不要当精确复发率。

3.2 动脉血栓栓塞（突然后腿瘫）：同样的模式

250 只在英国普通诊所处理的猫动脉血栓栓塞病例中，**153 只 (61.2%) 在初诊时即被安乐死**。而在**获得治疗机会**的猫里，**68/97 (70.1%) 存活超过 24 小时** (Borgeat 2014, PMID 24237457)。

⚠️ **这不是在说不该安乐死。** 这个病极度疼痛、预后确实差，安乐死常常是正确且仁慈的选择。**它是在说：这个决定经常在最初几分钟内做出，而当时没有人替这位主人量化过预后。**

3.3 一个便宜、可以直接问的检查

突然后腿瘫时，**测患肢与全身的静脉血糖差**可以把血栓和神经/骨科原因区分开。 ΔGlu 以 **30 mg/dL** 为界，在猫中**敏感度 100%、特异度 90%** (Klainbart 2014, PMID 25041102)。

两次血糖仪读数，不用麻醉、不用影像。

⚠️ 22 只血栓猫 vs 20 只对照，单中心、样本小，**不能替代主治兽医的判断**。写在这里是因为它便宜、快，而且是主人可以问出口的一个具体问题。

⚠️ 第 4 节 · 三件我必须对你说实话的事

4.1 「不吃多久算危险」——这个数字不存在

猫长时间不进食会引发**肝脂沉积**，这是一个独立的、致死率不低的急症。但**文献里只说"prolonged anorexia"（长时间厌食），从来没有量化过是几天。**

所以：**一只停止进食的猫应该被看，我不会给你一个"可以等 N 天"的阈值——因为那个数字是编的。**

4.2 「张口呼吸」——教条正确，但没有数据

猫（不像狗）不会良性地喘气，这是标准的兽医教学内容。但本项目从三个角度检索过，找不到任何量化研究。

所以：这条警告照给，但我不会说"研究表明"，因为没有。

4.3 没有任何单一数值能替你判断

71 只因呼吸窘迫送急诊的猫，入院乳酸值与存活、住院时长、生命体征、病因均无关联（Gilday 2022, PMID 36176706）。

不要让任何一个数字——乳酸、呼吸次数、体温——替代整体判断，也不要其中任何一个来安慰自己。这条同样适用于你在家数呼吸。

🤔 第 5 节 · 「我是不是太紧张了？」

这可能是你此刻最真实的顾虑。有两件事值得知道。

第一，人医的证据显示：症状自查工具的误差方向是「过度就医」，不是「延误」。也就是说，这类工具让人更容易多跑一趟，而不是更容易在家等——**它没有制造延误**（Chambers 2019 等，见知识库 § 3.1）。

第二，但这条证据来自人，不是猫。猫看病自费、猫不会说哪里痛——**主人的基线本来可能就偏向"再等等"**。人医的误差方向能不能搬到猫身上，**没有人测过**。

⚠️ 所以本页不承诺「你不会白跑」。它承诺的是另一件事：**白跑一趟的代价，几乎总是低于晚到一次的代价。**

附录 A · 文献清单

本页所有数字均来自以下文献，PMID 与 DOI 可直接核对。

⚠️ 本清单由脚本从归档的 PubMed 记录生成，非手工录入。

- Gerber, et al. Guarded long-term prognosis in male cats with urethral obstruction. *J Feline Med Surg* 2008;10(1):16-23. PMID 17719255. [DOI](#)
- Fitzgerald. Lily toxicity in the cat. *Top Companion Anim Med* 2010;25(4):213-7. PMID 21147474. [DOI](#)
- Borgeat, et al. Arterial thromboembolism in 250 cats in general practice: 2004-2012. *J Vet Intern Med* 2014;28(1):102-8. PMID 24237457. [DOI](#)
- Klainbart, et al. Peripheral and central venous blood glucose concentrations in dogs and cats with acute arterial thromboembolism. *J Vet Intern Med* 2014;28(5):1513-9. PMID 25041102. [DOI](#)

- Beer. Severe anemia in cats with urethral obstruction: 17 cases (2002-2011). *J Vet Emerg Crit Care (San Antonio)* 2016;26(3):393-7. PMID 26748857. [DOI](#)
- Beeston, et al. Occurrence and clinical management of urethral obstruction in male cats under primary veterinary care in the United Kingdom in 2016. *J Vet Intern Med* 2022;36(2):599-608. PMID 35199370. [DOI](#)
- Di Pietro, et al. Treatment of Permethrin Toxicosis in Cats by Intravenous Lipid Emulsion. *Toxics* 2022;10(4). PMID 35448426. [DOI](#)
- Gilday. Prognostic value of lactate in cats presented in respiratory distress to the emergency room. *Front Vet Sci* 2022;9:918029. PMID 36176706. [DOI](#)
- Manchester, et al. Difficult catheterization and previous urethral obstruction are associated with lower urinary tract tears in cats with urethral obstruction. *J Am Vet Med Assoc* 2024;262(2):187-192. PMID 38244269. [DOI](#)

附录 B • 我明确查不到的事

以下是文献中真实的空白。任何人给你一个填补这些空白的数字，那都是编的。

1. 猫厌食多少天会发生肝脂沉积 — 检索过，无天数阈值
2. 张口呼吸在猫的量化预后意义 — 极普遍的临床教条，无发表数据
3. 鼻出血/失血多严重算急诊 — 无量化数据
4. 主人使用分诊工具后是否真的改善了结局 — 人医研究测的是准确度，不是结局；猫的完全未测
5. 转运本身的风险 — 有机制记载，但population 数据给出的死亡比值比是 1.3 (95% CI 0.37 - 4.51, p = 0.680)，不显著；不能量化

原文摘录 (source excerpts)

这一节是本页的证据本身。正文是解释，可以争论；下面这些句子是原文，逐字未经翻译或改动，并由 `tools/dr_drill.py leg1` 在每次提交时比对归档的 PubMed 记录。

PMID 17719255 • Gerber B 2008

Of the 22 cats with idiopathic urethral obstruction, eight (36%) re-obstructed after 3-728 days (median 17 days).

Of 10 cats with urolithiasis, three (30%) re-obstructed after 10, 13 and 472 days, respectively.

Of the seven cats with urethral plugs, three (43%) re-obstructed after 4, 34 and 211 days, respectively.

Recurrent signs of lower urinary tract disease including obstruction were common in cats with urethral obstruction (20/39; 51%) and occurred in the same frequency irrespective of the primary cause of the obstruction.

Recurrent obstruction (14/39; 36%) was the most common reason for euthanasia and was performed in 8/39 (21%) cats.

Forty-five male cats with urethral obstruction or lower urinary tract signs referable to urethral obstruction were included in the study.

PMID 21147474 · Fitzgerald KT 2010

As little as 2 leaves or part of a single flower have resulted in deaths.

It should be pointed out that the whole plant-petals, stamen, leaves, and pollen are toxic.

Prognosis is excellent if fluid diuresis is started before anuric renal failure has developed.

Successful treatment includes initiation of fluid diuresis before the onset of anuric renal failure.

⚠ This is a narrative review, not primary data. The "2 leaves" figure is reported without an attached citation in the abstract; retrieve the full text before quoting it as a measured threshold.

PMID 24237457 · Borgeat K 2013

Over 98 months, 250 cats were identified with ATE.

At presentation, 153 cats (61.2%) were euthanized, with 68/97 (70.1%) of the remaining cats (27.2% of the total population) surviving >24 hours after presentation.

Of these, 30/68 (44.1%) survived for at least 7 days.

For cats that survived ≥ 7 days, median survival time was 94 (95% CI, 42-164) days, with 6 cats alive 1 year after presentation.

Although 153/250 cats were euthanized at presentation, 6 cats survived >12 months. No factors were identified that predicted euthanasia on presentation.

Cats with ATE presenting to GP are usually euthanized at presentation, but survival times >1 year are possible.

PMID 25041102 · Klainbart S 2014

Δ Glu cutoffs of 30 mg/dL and 16 mg/dL, in cats and dogs, respectively, corresponded to sensitivity and specificity of 100% and 90% in cats, respectively, and 100% in dogs.

Client-owned cats and dogs were divided into 3 respective groups: acute limb paralysis because of ATE (22 cats and 9 dogs); acute limb paralysis secondary to orthopedic or neurologic conditions (nonambulatory controls; 10 cats and 11 dogs); ambulatory animals presented because of various diseases (ambulatory controls; 10 cats and 9 dogs).

Δ Glu and % Δ Glu are accurate, readily available, diagnostic markers of acute ATE in paralyzed cats and dogs.

PMID 26748857 · Beer KS 2016

Cats in the UO-A group had a significantly longer duration of clinical signs ($P = 0.001$), and were more likely to have a history of previous urethral obstruction ($P = 0.011$), have a heart murmur ($P = 0.002$), have a gallop rhythm ($P = 0.005$), and have lower blood pressure ($P = 0.007$) compared to those in the UO group.

All UO cats survived to discharge, whereas 4/17 (23.5%) UO-A cats were euthanized ($P = 0.013$).

Seventeen cats with urethral obstruction and severe anemia (group "UO-A") that required transfusion were identified via medical record database search. Thirty cats with urethral obstruction and mild or no anemia (group "UO") were included as controls.

A history of previous urethral obstruction and longer duration of clinical signs may be important risk factors for severe anemia in UO cats.

PMID 35199370 · Beeston D 2022

From the study cohort of 237 825 male cats, there were 1293 incident cases.

The estimated UO incidence risk during 2016 was 0.54 (95% CI: 0.51-0.57).

Death during a UO episode was 329/1108 (29.6%), and 285/329 (88.0%) deaths involved euthanasia.

Overall repeat catheterization rate was 253/854 (29.6%).

Antibiotics were administered to 641/1108 (57.9%) cases.

Antibiotics were commonly prescribed in cats for treatment of UO despite minimal evidence in the clinical records of bacterial cystitis.

Repeat catheterization was common and case fatality rate during a UO episode was high.

PMID 35448426 · Ricci E 2022

In this study, the clinical response to treatment with intravenous lipid emulsion (ILE) for nine cats intoxicated with permethrin has been described.

The enrolled cats showed acute onset of seizures, tremors and hypersalivation that were partially controlled with the administration of benzodiazepines and intravenous fluid therapy.

After ILE treatment, all cats recovered rapidly and were discharged within 24-54 h.

⚠️ Nine cats, no control group. Cited for the clinical picture, not as evidence of treatment efficacy.

PMID 36176706 · Gilday C 2022

Admission lactate concentration was not associated with survival, duration of hospitalization, vital parameters, or underlying etiology for respiratory distress.

In contrast, lactate clearance was significantly associated with survival and length of hospitalization.

Seventy-one cats presented in respiratory distress to the ER at a university teaching hospital were enrolled in this retrospective study.

PMID 38244269 · Manchester RB 2023

The prevalence of lower urinary tears was 0.92% in UO cats.

Cats with lower urinary tears were significantly less likely to survive to discharge and had a longer period of hospitalization than cats without tears.